

Pre-Travel Health Advisory

United States · 7 days · Leisure

Departure: 5 December 2026

Traveller: Dike Wisdom

Low risk

COMPLETED

Apr 24, 2026

TRIP AT A GLANCE	
DURATION	7 days
PURPOSE	Leisure
TRAVELLING	Alone
ACCOMMODATION	Hotel (urban setting)
INSURANCE	Yes

ITINERARY-SPECIFIC GUIDANCE		
Trip type: ONE_WAY		
This is a one-way trip from Nairobi, Kenya to Oklahoma, United States, departing 5 December 2026. The recorded trip duration is 7 days; however, the questionnaire indicates an intended stay of 1 to 3 months. This discrepancy should be clarified with your physician before travel, as it affects medication supply, insurance coverage, and healthcare access planning.		
ROUTE GUIDANCE BY STOP		
STOP	COUNTRY	GUIDANCE
Oklahoma	United States	On arrival in Oklahoma in December, be prepared for cold weather — temperatures can be near or below freezing, particularly at night. Confirm your accommodation and have your travel insurance documents and emergency contacts accessible. Register your presence with the Nigerian Embassy or Consulate if staying for an extended period. Locate the nearest hospital or urgent care centre to your accommodation before you need it. Begin tick and insect precautions from your first outdoor activity. If you develop fever or feel unwell at any point, seek medical attention promptly and inform the clinician of your recent travel from Kenya.

HEALTH RISK OVERVIEW		
CATEGORY	LEVEL	SUMMARY
Food and water safety	MODERATE	Oklahoma has a safe municipal water supply and regulated food standards. Risk is low for standard hotel and restaurant dining. However, the departure point of Nairobi carries moderate food and water risk; travellers transiting or spending time there before departure should apply standard food and water precautions.
Vector-borne	MODERATE	Oklahoma is not a malaria-endemic region. However, West Nile virus is

diseases		present seasonally in Oklahoma and across the United States, transmitted by mosquitoes. Tick-borne illnesses including Rocky Mountain spotted fever and ehrlichiosis are also reported in Oklahoma, particularly relevant given planned hiking and outdoor activities. Standard insect and tick protection is advised.
Respiratory infections	MODERATE	Seasonal influenza circulates in the United States during December. COVID-19 remains a background risk in crowded indoor settings. Volunteering and humanitarian activities may increase exposure to respiratory pathogens. Up-to-date vaccination and standard respiratory hygiene are recommended.
Environmental health (heat, sun, air quality)	LOW	Oklahoma in December is typically cold with temperatures that can drop below freezing. Cold weather precautions including appropriate clothing and awareness of hypothermia risk during outdoor activities are more relevant than heat or sun exposure at this time of year. Air quality in Oklahoma City and Tulsa is generally acceptable.
Injuries and road traffic safety	MODERATE	Road traffic injury is a leading cause of harm for travellers in the United States. Oklahoma has significant road distances between destinations and driving is the primary mode of transport. Hiking and outdoor activities carry inherent injury risk. Dike is travelling alone, which increases the importance of having a clear emergency contact plan.
Rabies and animal contact	MODERATE	Rabies is present in wildlife in Oklahoma, including bats, raccoons, skunks, and foxes. Dike has indicated planned farm and animal contact activities. This elevates the risk of potential animal exposure. Pre-exposure rabies prophylaxis is recommended given the combination of animal contact activities and a destination where wildlife rabies is documented.
Blood-borne and sexual health	HIGH	Dike has indicated anticipated casual sexual relationships, inconsistent barrier protection (sometimes), and a prior history of sexually transmitted infection (STI). This combination represents a meaningful risk for HIV, hepatitis B, and other STIs. Consistent condom use, hepatitis B vaccination status verification, and awareness of PrEP and PEP availability in Oklahoma are strongly recommended.
Altitude-related illness	LOW	Oklahoma is a low-elevation state with a maximum elevation of approximately 1,516 metres at Black Mesa. This is well below the 2,500-metre threshold at which altitude sickness becomes clinically relevant. No altitude-related precautions are required for this trip.

FLIGHT & JOURNEY HEALTH

VTE Risk Level: MEDIUM

PREVENTION MEASURES

- The longest flight leg is 6 hours, which places Dike in the medium VTE (venous thromboembolism, or blood clot) risk category.
- Wear graduated compression stockings (15 to 30 mmHg) for the duration of the flight.
- Stay well hydrated throughout the flight — drink water regularly and avoid excessive alcohol and caffeine, which promote dehydration.
- Perform in-seat exercises every 1 to 2 hours: ankle circles, calf raises, and standing stretches in the aisle when safe to do so.

- Avoid prolonged immobility — get up and walk the aisle at least once every 2 hours.
- No additional pharmacological VTE prophylaxis (such as low molecular weight heparin) is indicated in the absence of additional VTE risk factors, but discuss with your physician if you have any concerns.

MEDICATION TIMING GUIDANCE

Dike is not currently taking any regular medications. No medication timing adjustments are required for this flight. If any medications are prescribed before departure (such as vaccines or prophylaxis), carry all medications in original packaging in hand luggage.

VACCINATIONS

VACCINE	STATUS	GUIDANCE
Routine immunizations (e.g. MMR, varicella, dTdap, polio /IPV)	Believed up to date	Recommended: verify documentation before travel Dike believes his routine vaccinations are up to date but has no documented travel vaccines. Confirm MMR (2-dose series), varicella, dTdap (tetanus-diphtheria-pertussis booster if more than 10 years since last dose), and polio/IPV are current. Given planned farm, animal, and volunteering activities, a tetanus booster is particularly important if not received within the past 5 years. Obtain written records from your healthcare provider.
Influenza	Unknown	Strongly recommended Annual influenza vaccination is recommended for all travellers to the United States. December falls within peak influenza season in the Northern Hemisphere. Dike's planned volunteering activities increase exposure risk. Administer before departure if not received this season.
COVID-19	Unknown	Recommended Ensure COVID-19 vaccination is up to date with the most current booster available. Check United States entry requirements for any current COVID-19 documentation requirements before departure. Respiratory hygiene in crowded indoor settings remains important.
Hepatitis A	Unknown — no travel vaccines received	Recommended Hepatitis A vaccination is recommended for travellers from Nigeria to the United States as a general travel health measure, and is particularly relevant given Dike's departure from Nairobi, Kenya, where hepatitis A risk is higher. A single dose provides protection for up to 1 year; a booster at 6 to 12 months provides long-term immunity. Administer at least 2 weeks before departure if possible.
Hepatitis B	Unknown — no travel vaccines received	Strongly recommended Given Dike's indicated casual sexual relationships, inconsistent condom use, and prior STI history, hepatitis B vaccination is strongly recommended if not previously completed. An accelerated schedule (doses at day 0, day 7, and day 21 to 28, with a booster at 12 months) is available if time before departure is limited. Confirm immunity status with a blood test if vaccination history is uncertain.

Typhoid	Not indicated for this destination	Not routinely recommended Oklahoma, United States has a treated municipal water supply and regulated food safety standards. Typhoid vaccination is not routinely recommended for travel to the United States. No action required for this destination.
Yellow fever	Not required or recommended	Not indicated Yellow fever vaccination is not required or recommended for travel to Oklahoma, United States. The United States is not a yellow fever-endemic country. Note: if Dike is transiting through or departing from Kenya, some countries require proof of yellow fever vaccination for travellers arriving from Kenya. Verify transit country requirements if applicable.
Japanese encephalitis	Not indicated	Not recommended Japanese encephalitis is not present in the United States. No vaccination required or recommended for this destination.
Meningococcal	Unknown	Consider — discuss with physician Meningococcal disease (MenACWY) is not a routine travel requirement for Oklahoma. However, given Dike's planned volunteering and humanitarian activities, which may involve close contact with groups of people, discuss with your physician whether MenACWY is appropriate. MenB may also be considered depending on the nature of the volunteering environment.
Rabies pre-exposure	Not received	Recommended — given planned animal and farm contact Rabies is present in wildlife in Oklahoma including bats, raccoons, skunks, and foxes. Dike has indicated planned farm and animal contact activities. Pre-exposure prophylaxis (PrEP) consists of 3 doses on days 0, 7, and 21 or 28. PrEP does not eliminate the need for post-exposure treatment but simplifies it significantly and removes the need for rabies immunoglobulin. Given the animal contact activities planned, this is recommended. Discuss with your travel medicine physician before departure.
Cholera (oral vaccine)	Not indicated	Not recommended Cholera is not a risk in Oklahoma, United States. The oral cholera vaccine is not recommended for this destination.

MALARIA PREVENTION

Risk Level: **NOT_INDICATED**

RECOMMENDED CHEMOPROPHYLAXIS

None required

Oklahoma, United States is not a malaria-endemic region. Malaria chemoprophylaxis is not indicated for this trip. No antimalarial medication is recommended.

MOSQUITO PROTECTION MEASURES

- Use EPA-registered insect repellents containing DEET (20 to 50 percent), picaridin, or IR3535 on exposed skin

	when outdoors, particularly during dawn and dusk when mosquitoes are most active.
	Wear long-sleeved shirts and long trousers when hiking or in areas with high mosquito or tick activity.
	Perform thorough tick checks after all outdoor and hiking activities. Remove any attached ticks promptly using fine-tipped tweezers.
	West Nile virus is seasonally present in Oklahoma. While December is lower risk due to cold temperatures reducing mosquito activity, insect precautions remain advisable during warmer periods of the stay.
CONTRAINDICATIONS / ALTERNATIVES	Not applicable — no chemoprophylaxis indicated.

MEDICAL ADVICE & RECOMMENDATIONS	
TOPIC	DETAILS
Pre-travel review and vaccination records	Book a pre-travel health consultation with your physician or a travel medicine clinic as soon as possible before your 5 December 2026 departure. Bring any existing vaccination records. Priority vaccines to address before departure include hepatitis A, hepatitis B (accelerated schedule if needed), influenza, COVID-19 booster, rabies pre-exposure prophylaxis, and confirmation of routine vaccines including tetanus. Given the 7-day duration and the range of activities planned, early consultation is important to allow adequate time for multi-dose vaccine schedules.
Food and water hygiene	Oklahoma has a safe municipal water supply. Tap water in hotels and restaurants is safe to drink. Standard food safety practices apply: choose freshly prepared, hot food; avoid raw or undercooked meat; wash hands thoroughly before eating. The higher food and water risk applies to the departure point in Nairobi, Kenya — apply strict food and water precautions there before boarding your flight.
Vector bite prevention	Oklahoma has documented tick-borne illnesses including Rocky Mountain spotted fever, ehrlichiosis, and tularaemia, which are relevant given planned hiking and outdoor activities. Use DEET-based or picaridin repellents on exposed skin. Wear long sleeves and trousers tucked into socks when hiking. Perform a full-body tick check after every outdoor activity. Remove attached ticks promptly. West Nile virus is also present seasonally; mosquito precautions apply particularly in warmer weather.
Sun, heat, and environmental precautions	Oklahoma in December is typically cold, with temperatures potentially below freezing, particularly at night. Dress in warm, layered clothing for outdoor and hiking activities. Be aware of wind chill, which can significantly lower the effective temperature. Hypothermia and frostbite are risks during prolonged outdoor exposure in cold conditions. Carry waterproof and windproof outer layers. Sun protection (SPF30 or higher) is still advisable on clear winter days, particularly at higher elevations during hiking.
Injury and road safety	Road travel is the primary mode of transport in Oklahoma. Always wear a seatbelt. If driving, familiarise yourself with local traffic laws. Oklahoma roads can become hazardous in winter due to ice and snow — exercise caution and check weather forecasts before driving. For hiking activities, inform someone of your planned route and expected return time. Carry a charged mobile phone, water, and a basic first aid kit. As a solo traveller, having a clear emergency contact plan is essential.

Sexual health and blood exposure	Dike has indicated anticipated casual sexual relationships, inconsistent use of barrier protection, and a prior history of STI. Consistent and correct condom use with all new sexual partners is strongly recommended to reduce the risk of HIV, gonorrhoea, chlamydia, syphilis, and hepatitis B. PrEP (pre-exposure prophylaxis for HIV) is widely available in the United States and can be discussed with a physician before or during the trip. PEP (post-exposure prophylaxis) is available in US emergency departments and must be started within 72 hours of a potential HIV exposure. Ensure hepatitis B vaccination is completed before departure. A pre-travel STI screen is advisable given the prior STI history. Post-travel STI testing is also recommended on return.
Jet lag, sleep, and mental wellbeing	The time difference between Nairobi (East Africa Time, UTC+3) and Oklahoma (Central Standard Time, UTC-6) is 9 hours. This is a significant time zone shift and jet lag is likely. Strategies to manage jet lag include adjusting sleep and meal times toward the destination time zone in the days before departure, staying hydrated during the flight, avoiding excessive alcohol, and seeking natural daylight on arrival. As a solo traveller, be mindful of the psychological adjustment to a new environment. Alcohol consumption, which Dike has indicated is anticipated, can worsen jet lag, impair judgment, and reduce medication adherence — moderate intake is advised.
Malaria and other chemoprophylaxis (state if not indicated)	Malaria chemoprophylaxis is NOT indicated for Oklahoma, United States. The United States is not a malaria-endemic country. No antimalarial medication is required for this trip. Note: if Dike has been in a malaria-endemic region such as Kenya prior to departure, and develops fever or flu-like symptoms after arrival in Oklahoma, malaria should be considered and medical attention sought promptly, as symptoms can appear days to weeks after leaving an endemic area.
Traveller-specific considerations (from health context)	Several factors specific to Dike's profile require particular attention. First, Dike has no documented travel vaccines and has not previously visited a travel clinic — a comprehensive pre-travel consultation is essential before this trip. Second, the combination of volunteering and humanitarian activities with farm and animal contact in a rabies-present country makes rabies pre-exposure prophylaxis a meaningful recommendation. Third, the anticipated alcohol consumption combined with casual sexual relationships and inconsistent condom use creates a compounding risk profile for STIs and HIV — these risks are directly linked and should be addressed together in the pre-travel consultation. Fourth, Dike is travelling alone on a one-way ticket with a stated stay of 1 to 3 months despite the trip being recorded as 7 days — clarify the actual intended duration with your physician, as a longer stay would significantly change medication supply requirements, insurance coverage needs, and healthcare access planning in the United States.

MEDICATION LOGISTICS	
PACKAGING	Carry all medications and vaccines in original manufacturer packaging with pharmacy labels intact. Obtain a physician letter listing all medications by generic and brand name for airport security and customs.
SUPPLY RULE	Carry a minimum of double the required supply split between hand luggage and checked baggage. Given the potential for a stay of 1 to 3 months (as indicated in

	the questionnaire), ensure any prescribed medications cover the full intended duration.
DESTINATION LEGALITY VERIFIED	Yes — confirm all medications are legal at destination.

CLINICAL DECISION FLAGS
▪ TREE_3_MEDIUM_RISK_DESTINATION
▪ TREE_4_DURATION
▪ TREE_5_FLIGHT
▪ TREE_9_ANIMAL_FARM_CONTACT: Rabies pre-exposure prophylaxis recommended — rabies present in Oklahoma wildlife
▪ TREE_9_VOLUNTEERING_HUMANITARIAN: Bloodborne virus risk assessment required; hepatitis B status mandatory
▪ TREE_13_NO_PRIOR_TRAVEL_VACCINES: Full vaccine catch-up required; first travel clinic visit
▪ TREE_14_CASUAL_RELATIONSHIPS_INCONSISTENT_PROTECTION_STI_HISTORY: Elevated STI and HIV risk; hepatitis B vaccination; PrEP/PEP counselling required
▪ TREE_14_ALCOHOL_CONSUMPTION: Impaired judgment risk affecting medication adherence, sexual safety, and accident risk
▪ ITINERARY_DISCREPANCY: Questionnaire states 1 to 3 month stay; current trip records 7 days — clarify actual duration before travel

SPECIALIST REFERRALS		
CONDITION	SPECIALIST	URGENCY
No prior travel vaccines and multiple risk activities planned including animal contact, volunteering, and casual sexual activity	Travel medicine physician or travel health clinic	BEFORE_TRAVEL
Rabies pre-exposure prophylaxis assessment — planned farm and animal contact in rabies-present destination	Travel medicine physician	BEFORE_TRAVEL
Hepatitis B vaccination and sexual health risk assessment — prior STI history, inconsistent condom use, anticipated casual relationships	Travel medicine physician or sexual health clinic	BEFORE_TRAVEL
Itinerary discrepancy — questionnaire indicates 1 to 3 month stay versus 7-day trip record; medication supply, insurance, and healthcare access planning depend on accurate duration	Primary care physician	BEFORE_TRAVEL

SEXUAL HEALTH & RISK BEHAVIOURS
Risk Level: HIGH
PREVENTION ADVICE

	Use condoms consistently and correctly with all new or casual sexual partners throughout the trip. Inconsistent use, as indicated, significantly increases risk of HIV, gonorrhoea, chlamydia, syphilis, and hepatitis B.
	Ensure hepatitis B vaccination is completed before departure — an accelerated 3-dose schedule is available if time is limited.
	Discuss PrEP (pre-exposure prophylaxis for HIV) with your physician before departure. PrEP is highly effective when taken as prescribed and is widely available in the United States.
	If a potential HIV exposure occurs, PEP (post-exposure prophylaxis) must be started within 72 hours. PEP is available at US hospital emergency departments. Do not delay seeking it.
	Alcohol consumption impairs judgment and is directly associated with inconsistent condom use and higher-risk sexual decisions. Be aware of this interaction.
	A pre-travel STI screen is recommended given the prior STI history. Post-travel STI testing on return is also strongly advised.
	Carry an adequate supply of condoms from home — quality and availability may vary.
PREP/PEP	PrEP/PEP availability discussed for high-risk destination.

AFTER YOU RETURN	
	RED FLAGS — SEEK IMMEDIATE CARE IF YOU EXPERIENCE Seek immediate medical attention if you develop: high fever (above 38 degrees Celsius) at any point after return, particularly within 3 months of leaving Kenya; a spreading skin rash with fever; severe headache with neck stiffness; difficulty breathing; signs of a serious wound infection; or any symptoms following an animal bite or scratch. Always inform the treating clinician of your recent travel history including both Kenya and the United States.
WITHIN 1 WEEK	
	Monitor for fever, chills, or flu-like symptoms. Although malaria is not a risk in Oklahoma, if Dike spent time in Kenya before departure, malaria can present weeks after leaving an endemic area — seek medical attention promptly if fever develops and mention recent travel to Kenya.
	Check for any tick bites or skin reactions following hiking or outdoor activities. Early signs of tick-borne illness include fever, headache, rash, and muscle aches — seek medical attention if these develop.
WITHIN 4 WEEKS	
	Attend a post-travel health check with your primary care physician, particularly given the volunteering and animal contact activities undertaken.
	Undergo STI screening including HIV, gonorrhoea, chlamydia, syphilis, and hepatitis B if any unprotected or inconsistently protected sexual contact occurred during the trip.
	Report any unexplained rash, gastrointestinal illness, or respiratory symptoms to your physician and mention your travel history.
	If rabies pre-exposure prophylaxis was not completed before travel and any animal contact or bite occurred, seek urgent medical advice immediately — do not wait.
BEYOND 4 WEEKS	
	Complete the hepatitis B vaccine series if an accelerated schedule was started before departure.

- Complete the rabies pre-exposure prophylaxis series if started before travel.
- If any concerns about STI exposure remain, repeat testing at 3 months post-exposure for HIV provides a definitive result.
- Review travel insurance claims and any medical care received during the trip with your insurer.

MEDICAL CARE & CONTACTS

CLINICS & FACILITIES

NAME	LOCATION & NOTES
OU Health University of Oklahoma Medical Center	700 NE 13th Street, Oklahoma City, OK 73104 +1 405 271 4700 · Oklahoma City — primary tertiary referral centre Major academic medical centre with emergency and specialist services. Travel medicine consultation may be available through infectious disease department.
Tulsa Travel Medicine and Immunization Clinic	Tulsa, Oklahoma — confirm current address via CDC travel clinic finder at wwwnc.cdc.gov/travel/page/find-clinic Confirm via CDC clinic finder · Tulsa area Use the CDC travel clinic finder to locate the nearest accredited travel medicine clinic in Oklahoma before and during your trip.

EMBASSY & CONSULAR

NAME	DETAILS
Nigerian Embassy in Washington DC (nearest Nigerian diplomatic mission to Oklahoma)	3519 International Court NW, Washington DC 20008. Phone: +1 202 986 8400. Website: nigeriaembassyusa.org . Nigerian citizens in Oklahoma may also contact the Nigerian Consulate General in Atlanta: +1 404 765 9947.

EMERGENCY NUMBERS

LABEL	CONTACT
US Emergency Services (Police, Fire, Ambulance)	911
US Poison Control Center	1 800 222 1222
CDC Travellers Health Information Line	1 800 232 4636

NEXT STEPS

- Book a pre-travel health consultation with a travel medicine physician or clinic as soon as possible — ideally at least 4 to 6 weeks before 5 December 2026 to allow time for multi-dose vaccine schedules.
- Prioritise the following vaccines at your consultation: hepatitis A, hepatitis B (accelerated schedule), influenza, COVID-19 booster, rabies pre-exposure prophylaxis, and confirmation of routine vaccines including tetanus /dTdap.
- Discuss PrEP (HIV pre-exposure prophylaxis) with your physician given the anticipated sexual risk behaviours.
- Undergo a pre-travel STI screen given your prior STI history.

- Clarify your actual intended length of stay (7 days versus 1 to 3 months) with your physician — this affects insurance, medication supply, and healthcare access planning in the United States.
- Ensure your travel insurance policy covers the full intended duration of stay, includes medical evacuation, and covers pre-existing conditions and any activities planned including hiking and volunteering.
- Purchase and pack an adequate supply of condoms for the trip.
- Obtain a physician letter and carry all medications and vaccines in original packaging for airport security.
- Familiarise yourself with how to access PEP (HIV post-exposure prophylaxis) in Oklahoma — it is available at hospital emergency departments and must be started within 72 hours of a potential exposure.
- Perform a full-body tick check after every hiking or outdoor activity during your stay.

This advisory has been generated by the TMAG (Travel Medicine Advisory Global) AI Clinical Decision Support System and is intended to inform a pre-travel health consultation with a qualified clinician. It does not constitute medical advice and must not replace a consultation with a licensed physician, travel medicine specialist, or pharmacist. All vaccine and medication recommendations require clinician review and a valid prescription where applicable. Information is based on CDC and WHO travel health guidelines current at the time of generation; destination risk profiles may change. Always check for the latest alerts before departure. Travellers with complex medical histories, chronic conditions, or pregnancy are strongly advised to seek specialist travel medicine review in person before travelling. Medication guidance in this report is advisory only and must not be used to alter, substitute, or discontinue any medication currently prescribed to you. Please consult your physician or pharmacist before acting on any medication recommendation. Copyright 2026 Travel Medicine Advisory Global. travelmedicine.global

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DOCTOR VERIFICATION & DIGITAL SIGNATURE

This travel health plan has been reviewed and approved by a licensed physician on the TMAG Doctor Network.

PHYSICIAN NAME	Mat Smith
LICENSE NUMBER	MT898320084
VALIDATION DATE	24 April 2026
STATUS	TMAG Verified & Approved



Digital Signature

TMAG Verified Seal

This document was generated by Travel Medicine Advisory Global and approved by a licensed physician.